INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Sunitinib 37.5 mg · pancreatic neuroendocrine tumour (NET)

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00327
Version 5

Continuous daily capsules

Treatment stages

01 **Continuous daily sunitinib**
28-day review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call urgently for severe diarrhoea or dehydration, bleeding, new swelling, much less urine, severe skin blistering or symptoms of low blood sugar. Chest pain, severe breathlessness, collapse, seizure or severe headache with vision changes needs emergency assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

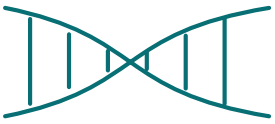
How your medicines work

Sunitinib is a daily targeted capsule used to help control a pancreatic NET that cannot be removed or has spread and is progressing. The team reviews control, side effects and your priorities.

TARGETED THERAPY CAPSULE

Sunitinib

Sutent



Blocks several growth and blood-vessel signals

Sunitinib blocks several signalling proteins involved in tumour growth and blood-vessel formation. This can slow a pancreatic NET. Effects on healthy blood vessels and other tissues help explain high blood pressure, bleeding, wound-healing problems, thyroid changes and sore hands or feet.

Growth / vessel signals → Receptors blocked → Growth may slow

What to watch for

Keep blood-pressure and blood-test appointments; report bowel, skin, heart or bleeding symptoms early.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your sunitinib treatment

This guide is for the continuous pancreatic NET regimen.

What the trial can tell us

Sunitinib delayed progression compared with placebo in this population. The trial stopped early; early survival findings require caution and do not predict one person's lifespan. The results apply to the pancreatic NET population and continuous dose studied, not every NET site or another sunitinib schedule.

Understanding your particular NET

Several different features help the team choose treatment.

Where it started and where it is now

A neuroendocrine tumour begins in cells with hormone-making features. Its starting organ and the places it has spread are separate parts of the diagnosis. A pancreatic neuroendocrine tumour differs from pancreatic adenocarcinoma; a deposit in the liver may have come from another organ.

Differentiation, grade and Ki-67

The pathology report describes how organised the cells look (differentiation) and their growth activity (grade). Ki-67 measures cells showing a growth-related protein in the sample. Its percentage describes that sample, not the amount of your body affected or your chance of survival.

Grade is different from stage

Stage describes extent; grade describes tumour biology. Either can vary independently. A well-differentiated grade 3 neuroendocrine tumour and a poorly differentiated neuroendocrine carcinoma are distinct diagnoses and may need different treatments.

Functioning and nonfunctioning

A functioning tumour produces a hormone syndrome. A nonfunctioning tumour can still cause problems through growth or pressure. Somatostatin receptor uptake on a scan shows a treatment target; it does not, on its own, identify a hormone syndrome.

Your diagnosis record

Ask for one diagnosis summary: starting organ, grade, differentiation, Ki-67 result if reported, sites involved, hormone syndrome and receptor-imaging result. Keep pending results clearly marked.

Your overall plan and choices

Your priorities belong in the treatment plan.

What are we trying to achieve?

Agree your treatment aim: slowing growth, reducing tumour size, controlling hormone symptoms or preserving daily function. A scan showing no growth may be a useful result even when the tumour has not shrunk.

How treatments fit together

Your plan may use different treatments at different times, including surgery, medicines, receptor-targeted radiation or treatment directed at the liver. Ask the team to mark your own sequence and who coordinates it.

Choosing between options

Ask why the proposed option fits the tumour site, grade, receptor findings, symptoms and your health. Discuss reasonable alternatives, observation when appropriate and suitable trials. Your team explains which options are available to you.

Evidence is about groups

Studies measure different outcomes, such as shrinkage, time before worsening and survival. Ask which outcome matters for your plan and how closely the study group matches your diagnosis. No study average can predict one person’s course.

Review points and shared decisions

Agree when to review the plan and what could lead to continuing, pausing or changing treatment. Include the effects on work, caring duties, travel and the activities that matter to you.

Evidence for this continuous pancreatic NET schedule

The randomised trial compared daily sunitinib with placebo, with supportive care.

CHAPTER 02

Your schedule, tests and appointments

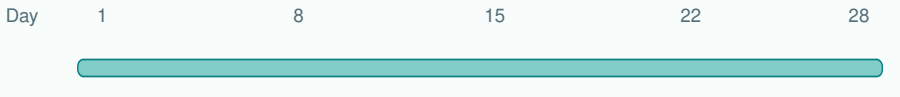
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

How your treatment fits together

Sunitinib 37.5 mg · pancreatic NET

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY REVIEW

Continuous daily sunitinib

Daily | Review

This is the 37.5 mg continuous pNET schedule, not a four-weeks-on/two-weeks-off schedule. Continue only as authorised by the team.

The written prescription and team review determine treatment; this timeline is a reference.

BASELINE → TREATMENT → FOLLOW-UP

Treatment safety, NET response and follow-up

Your team books and reviews each test. Response scans and hormone or tumour markers follow an individual plan; they are not automatically required at every medicine review.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00327 v5 · current regimen · EMA product information and patient leaflet

Tests in your plan

5-hydroxyindoleacetic acid (5-HIAA)

If part of your plan

WHEN

Only if requested for your hormone syndrome; the team sets the dates.

Measures a breakdown product of serotonin to help assess selected neuroendocrine tumours.

Often a timed urine collection. Follow the laboratory's food and medicine instructions.

Chromogranin A

If part of your plan

WHEN

Only when the team considers it informative for your tumour.

A selected tumour-marker result, interpreted with symptoms and scans.

Acid-suppressing medicines and other conditions can alter the result. It is not a routine test before every treatment; do not stop medicines yourself.

Selected pancreatic hormones

If part of your plan

WHEN

Only for the hormone question identified by your specialist.

Examples include gastrin, insulin or glucagon when the diagnosis or symptoms call for them.

The team names the test, preparation and review date; these are not a standard panel for every patient.

THE WHOLE TREATMENT JOURNEY

Sunitinib 37.5 mg · pancreatic NET: treatment and checks

Progressive unresectable or metastatic well-differentiated pancreatic neuroendocrine tumour under NCCP 00327a.

Take the prescribed sunitinib dose once daily continuously, at the same time, consistently with or without food. The 28-day cycle is a review period, with no planned capsule-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00327	Baseline tests	Before each 28-day cycle	Earlier tests if symptoms or dehydration	Review results before prescribing	Follow persistent abnormalities
Blood pressure / heart NCCP 00327	Baseline blood pressure (BP); cardiac assessment and echo/ multigated acquisition (heart pumping) scan (MUGA) if indicated	BP at every attendance; heart tests if indicated	Report chest pain, breathlessnes s or swelling	Agree management of BP or heart changes	Individual follow-up
Thyroid / urine / glucose NCCP 00327	Thyroid tests; baseline urine assessment; glucose review	Thyroid every 12 weeks; urine/glucose as clinically indicated	Report cold intolerance, marked fatigue or low-glucose symptoms	Review medicine effects and diabetes treatment	Follow unresolved changes

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
NET response NCCP 00327	Record symptoms, disease and personal goals	Consultant-directed imaging / markers	Track symptoms and adherence	Assess benefit and treatment burden	Plan next treatment or surveillance

What each check is for

1 **Continuous daily sunitinib**

28-day review

Sunitinib is a daily targeted capsule used to help control a pancreatic NET that cannot be removed or has spread and is progressing. The team reviews control, side effects and your priorities.

Ask / **act:** Confirm the current prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Capsules and missed doses

The usual starting dose for this pancreatic NET schedule is 37.5 mg once daily. Follow your label, swallow whole with water and take at the same time each day consistently with or without food. If a dose is missed, do not take an additional dose; take the usual prescribed dose the next day.

STEP 2

Interactions

Avoid grapefruit and St John’s wort. Ask the pharmacist to review every medicine and supplement, including blood thinners, antibiotics and medicines affecting heart rhythm.

STEP 3

Surgery and dental work

Tell your team about planned operations, wounds or invasive dental work. A treatment interruption may be needed for major surgery; the oncology and surgical teams agree when to stop and restart. Ask about a dental assessment.

STEP 4

Storage and supply

Keep capsules in their original labelled packaging, away from children, and follow storage instructions. Return unused medicine to the pharmacy. Bring your current prescription and medicine list to review.

STEP 5

Pregnancy, feeding and fertility

Tell the team about pregnancy, planned pregnancy or breastfeeding before treatment. Ask for the contraception and fertility plan specific to sunitinib, including how long precautions continue after stopping.

STEP 6

Tests and visits

Baseline blood count, kidney/liver, thyroid and blood pressure; urine protein assessment and cardiac tests as indicated. Counts, kidney and liver tests before each cycle; BP at each attendance; thyroid every 12 weeks. Heart, urine and glucose monitoring are individualised. Scans and tumour markers follow the consultant plan.

Taking capsules and keeping safe

Use your individual prescription and symptom plan.

Skin and mouth care

Use the unit’s gentle skin and mouth-care plan. Report hand-foot pain before it limits walking or daily tasks; the team may adjust treatment.

Sickness and diarrhoea

This oral regimen is minimal-to-low emetogenic. Routine premedication is not required. Agree rescue anti-sickness and antidiarrhoeal medicines and when to call.

Blood pressure and glucose

Follow the agreed BP plan. If you use diabetes medicines, ask how to monitor glucose and manage low readings; sunitinib may lower blood sugar.

Nutrition and fatigue

Report poor intake, weight loss or troublesome fatigue. Thyroid changes, anaemia, dehydration or other causes may need treatment.

Making sense of tests and results

Safety tests, tumour tests and hormone tests answer different questions.

Blood tests for treatment safety

Blood counts assess oxygen-carrying cells, infection-fighting cells and platelets. Kidney and liver results help assess treatment safety. These checks are needed even when you feel well; a reassuring scan answers a different question.



computed tomography (CT), magnetic resonance imaging (MRI) and receptor imaging

CT and MRI show anatomy and tumour size. Somatostatin receptor imaging tests whether a particular receptor target is present. An fluorodeoxyglucose positron emission tomography (FDG-PET) scan measures uptake of a glucose-like tracer and is used selectively. Your team chooses the scans that can guide your care.

Hormones and tumour markers are individual

Selected markers may help answer a hormone or tumour question. Examples include 5-hydroxyindoleacetic acid (5-HIAA) for serotonin-related symptoms, chromogranin A, or a specific pancreatic hormone. They are not a routine panel for every neuroendocrine tumour. Interpret changes with symptoms and imaging.

Preparing for a test

Use the laboratory instructions for fasting, food restrictions and any timed urine collection. Some medicines affect marker results, including acid-suppressing medicines and chromogranin A. Ask the team what to do; do not stop a prescribed medicine yourself.

Heart and inherited-risk checks

Some hormone syndromes can affect heart valves, so an ultrasound of the heart may be requested. Inherited-risk assessment depends on the tumour and family history; multiple endocrine neoplasia type 1 is one example. An inherited-risk test and a test of tumour tissue answer different questions.

Who does what, and when?

For each test, write its name, purpose, booking date and result-review contact. Ask the team to set your next scan or marker date. A schedule for someone after surgery may not apply during your treatment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Sunitinib every day	 **Days 1–28** No planned rest week.
Symptoms and blood pressure	 **Days 1–28** Report changes promptly.
28-day review	 **Day 28** Counts, kidneys/liver and prescription review; BP at every attendance.

Continuous does not mean unchecked: Take capsules daily only as prescribed; reviews, test results or toxicity can lead to a pause or dose change.

After a hold: Record the last dose and reason. Your oncology team decides when and at what dose to restart.

No borrowed rest week: Other sunitinib schedules use breaks. This pancreatic NET regimen is continuous; follow its own calendar.

> **Why these bands stay broad:** Treatment timing is shown for orientation. Symptoms can develop on any day; no symptom band predicts an individual toxicity peak. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00327 v5 · current regimen ↗

> **There are no guaranteed "safe days".** Follow your oncology unit's emergency instructions. Fever, shivering or suddenly feeling unwell needs urgent advice; do not wait for the next appointment. Call emergency services for collapse, severe breathing difficulty, chest pain or uncontrolled bleeding.

Between-treatment notes

Day 1–7: Start with a clear prescription

Confirm the continuous dose, blood-pressure plan, skin care and other medicines. Record any existing bowel, mouth, skin or heart symptoms.

Day 8–28: Keep taking daily unless told to hold

Use the written symptom plan and report changes early. A new review window does not create a break. Attend review bloods; thyroid tests are due every 12 weeks.

Support around the medicines

1 **Blood pressure and glucose**

Urgent symptoms and treatment changes

Call when a new problem develops.

Do not restart a held dose yourself

The team may pause, reduce or stop treatment for toxicity or test abnormalities. Record the reason and agreed restart plan; do not use another patient's dose or timetable.

Hormone symptoms and day-to-day changes

A new symptom may come from the NET, treatment or another illness.

Build a picture of what is normal for you

Keep a brief record of your usual bowel pattern, flushing, appetite, weight, pain and energy. Note new changes and when they started. This helps the team assess whether treatment, hormones or another problem needs attention.

Hormone-related symptoms

Hormone symptoms depend on the tumour: examples include flushing and watery diarrhoea, acid-related symptoms, or changes in blood sugar. An insulin-producing tumour can lower blood sugar. Ask which symptoms apply to you and whether you need a written rescue plan.

Hormone treatment is a separate part of the plan

Hormone-control medicines have their own schedule. A pause in anticancer tablets does not automatically mean pausing those medicines. Before peptide receptor radionuclide therapy, the specialist team coordinates any interruption and restart of somatostatin analogue injections.

When a change is urgent

Follow your oncology emergency plan for sudden or severe symptoms. Collapse, severe breathing difficulty, chest pain, confusion or uncontrolled bleeding needs emergency assessment. Severe diarrhoea, persistent vomiting, dehydration, fever or sudden illness needs urgent oncology advice. Do not wait for a scan, an online answer or the next clinic visit.

Procedures and hormone syndromes

Tell teams arranging an operation, anaesthetic or other procedure about a hormone-producing tumour or previous hormone crisis. Your specialist decides whether extra preparation is needed. Carry the agreed emergency instructions and current medicine list.

Eating, energy, practical help and support

Supportive care can be used alongside treatment from the start.

Food, weight and digestion

Report weight loss, persistent diarrhoea, greasy stools or difficulty eating. Different causes need different treatment. Ask for a dietitian and whether reduced digestive-enzyme function needs assessment.

Fluids and safe activity

Use your personal fluid advice, especially with diarrhoea or heart or kidney disease. Adjust activity and rest to your energy. Report dizziness, breathlessness or fatigue that limits daily tasks so treatable causes can be checked.

Emotions, relationships and quality of life

Ask for help with mood, sleep, relationships, intimacy or caring responsibilities when you need it. Symptom and palliative-care teams can support comfort and quality of life while anticancer treatment continues.

Practical help

Ask your specialist nurse or social worker about transport, employment, finances and help at home. Tell them about caring responsibilities and ask which support organisations fit your needs.

A record that works without a phone

Keep a paper schedule, current medicine list and real contact numbers together. The handbook can be read without internet access. Use the written emergency plan whenever you need urgent help.

Questions to take to the next visit

For the next visit, choose the questions that matter most: my exact diagnosis; the aim of treatment; which tests I need and why; who explains results; when to call; and what help is available.

Questions worth asking

Is my dose the continuous pancreatic NET schedule?

For this NCCP pancreatic NET regimen, the standard starting schedule is 37.5 mg once daily continuously, with 28-day reviews and no planned rest week. Your own prescribed dose may differ.

Who checks my BP and when are my 12-week thyroid tests?

Blood pressure is checked at each attendance and thyroid tests every 12 weeks, with extra tests if indicated. Ask who will book them and where to record readings.

What should I do about diarrhoea or painful hands and feet?

Report these symptoms early and follow the written rescue and hold plan. Painful hands or feet can require interruption even at the first episode; the team confirms dose changes and restarting.

Do my diabetes or blood-thinning medicines need extra monitoring?

Ask the pharmacist to check the full medicine list. Diabetes may require closer glucose checks and changes to diabetes treatment; bleeding risks and interactions need individual review.

What is the plan for dental work or surgery?

Tell the oncology team before arranging procedures. Surgery, wounds and dental work may need a coordinated interruption, with restart decided by the treating teams.

When will we review response and quality of life?

Ask for the planned symptom, scan and treatment-review dates. There is no fixed total number of cycles; continued treatment depends on benefit, tolerability and your priorities.

Try explaining the plan back: Can you explain why this is continuous daily dosing, where to record blood pressure and when painful hands/feet or diarrhoea need a call?

Your neuroendocrine tumour treatment Progressive unresectable or metastatic well-differentiated pancreatic neuroendocrine tumour under NCCP 00327a.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous daily sunitinib · pancreatic NET

Active medicines: Sunitinib

Emetogenic risk: Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Use the unit's gentle skin and mouth-care plan. Report hand-foot pain before it limits walking or daily tasks; the team may adjust treatment.
- This oral regimen is minimal-to-low emetogenic. Routine premedication is not required. Agree rescue anti-sickness and antidiarrhoeal medicines and when to call.
- Follow the agreed blood pressure (BP) plan. If you use diabetes medicines, ask how to monitor glucose and manage low readings; sunitinib may lower blood sugar.
- Report poor intake, weight loss or troublesome fatigue. Thyroid changes, anaemia, dehydration or other causes may need treatment.
- No routine premedication is required by NCCP. Any antidiarrhoeal, rescue antiemetic or other support must follow the local prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / thyroid / energy

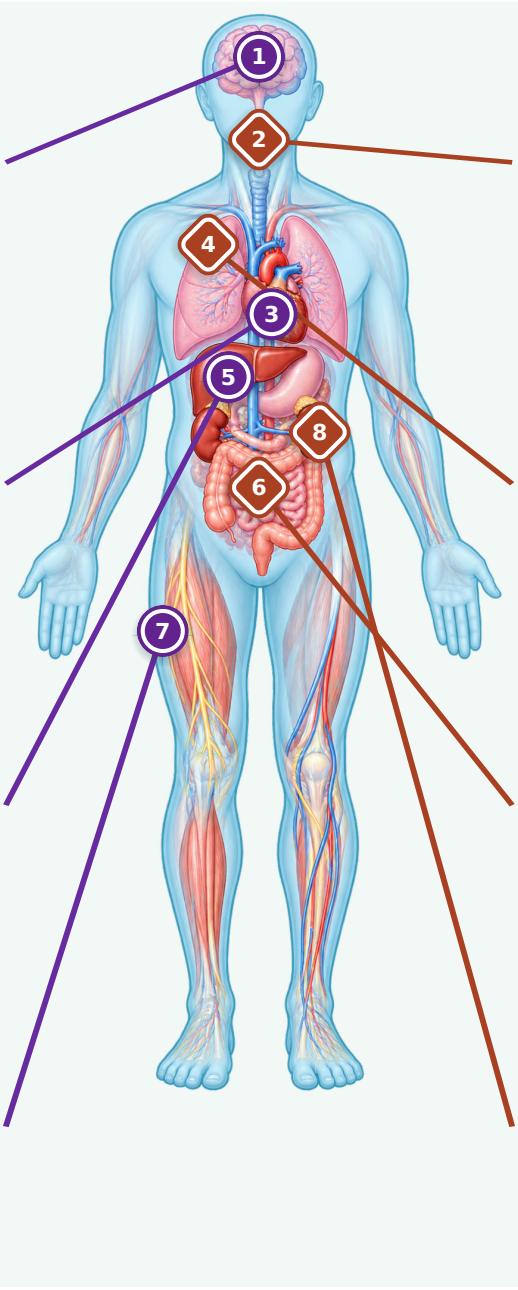
Sunitinib

Thyroid changes can contribute to tiredness. Severe headache, seizure, confusion or visual change needs emergency assessment.

2 Mouth / jaw

Sunitinib

Report mouth soreness, eating problems or jaw pain. Discuss dental work with your team before treatment or procedures.



3 Heart / blood pressure

Sunitinib

Blood pressure may rise without symptoms. Keep checks; new swelling or breathlessness needs advice, and chest pain needs emergency care.

4 Blood / bleeding

Sunitinib

Low counts can increase infection or bleeding risk. Report fever, sudden illness, unusual bruising or bleeding; heavy bleeding is an emergency.

5 Liver

Sunitinib

Liver tests may change before symptoms appear. Attend blood tests and report yellow skin or eyes, dark urine or severe abdominal pain.

6 Bowel / fluids

Sunitinib

Diarrhoea can cause dehydration. Follow the written plan and call for persistent symptoms, dizziness or inability to drink.

7 Hands / feet / skin

Sunitinib

Report painful redness, swelling or blisters early. Severe widespread blistering or peeling requires urgent help.

8 Kidneys / glucose

Sunitinib

Urine protein and kidney or glucose changes may need tests. Report reduced urine, swelling, shakiness, sweating or confusion.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for severe diarrhoea or dehydration, bleeding, new swelling, much less urine, severe skin blistering or symptoms of low blood sugar. Chest pain, severe breathlessness, collapse, seizure or severe headache with vision changes needs emergency assessment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **Use the prescribed local supportive-care plan; independent clinical/pharmacy review pending**
   https://healthservice.hse.ie/documents/6919/327_V5_SUNitinib_37.5mg.pdf
2. **EMA product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/sutent-epar-product-information_en.pdf
3. **NCI PDQ: gastrointestinal neuroendocrine tumours (NETs) · selected marker background**
   https://www.cancer.gov/types/gi-neuroendocrine-tumors/hp/gi-neuroendocrine-treatment-pdq
4. **NCI PDQ: pancreatic NETs · hormone and digestive background**
   https://www.cancer.gov/types/pancreatic/hp/pnet-treatment-pdq
5. **Raymond et al. phase III sunitinib pancreatic NET trial**
   https://pubmed.ncbi.nlm.nih.gov/21306237/
6. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
7. **NANETS expert consensus: high-grade neuroendocrine neoplasms, 2023**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10388681/
8. **NCI: gastrointestinal neuroendocrine tumours — patient information**
   https://www.cancer.gov/types/gi-neuroendocrine-tumors/patient/gi-neuroendocrine-treatment-pdq
9. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
10. **NCI: palliative care during cancer treatment**
    https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
11. **NCI: pancreatic neuroendocrine tumour diagnosis and tests**
    https://www.cancer.gov/types/pancreatic/neuroendocrine-tumors/diagnosis
12. **NCI: pancreatic neuroendocrine tumour symptoms**
    https://www.cancer.gov/types/pancreatic/neuroendocrine-tumors/signs-symptoms
13. **NCI: pancreatic neuroendocrine tumour treatment**
    https://www.cancer.gov/types/pancreatic/neuroendocrine-tumors/treatment
14. **NCI: pancreatic neuroendocrine tumour types**
    https://www.cancer.gov/types/pancreatic/neuroendocrine-tumors